

Standard Operating Procedure

Key Performance Indicator Data Collection, Verification and Analysis

«Hospital Name» — Supports PSQ.3.a-f

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By Quality Coordinator Name: _____ Signature: _____			
Approved By Medical Superintendent Name: _____ Signature: _____			

1. Purpose

This SOP describes how «Hospital Name» defines, collects, verifies and analyses key performance indicators (KPIs) covering clinical, infection-control, managerial and patient-safety structures, processes and outcomes.

2. Scope

Applies to every KPI the hospital monitors, whether mandated by government/NABH or chosen locally.

3. Responsibility

Quality Team: defines indicators, collects and verifies data, and analyses trends.

Department Heads: ensure accurate source data from their department.

4. Procedure

1. For every indicator, define the numerator (what is counted as an event), the denominator (the population or exposure it is measured against), and the multiplier (the scale the rate is expressed at, e.g. per 100 or per 1,000).

2. Worked example — CAUTI rate: numerator = number of catheter-associated urinary tract infections in the month; denominator = number of urinary-catheter-days in the same month; multiplier = 1,000. Rate = (numerator ÷ denominator) × 1,000, expressed as infections per 1,000 catheter-days. Use the current

CDC/National Healthcare Safety Network (NHSN) surveillance definitions to decide what counts as a CAUTI event, so results are comparable year to year. The same numerator/denominator/multiplier structure applies to CLABSI (central-line-days), VAP (ventilator-days), and SSI (procedures performed).

3. Collect data monthly (or as the indicator requires) from the source system or manual log.
4. Verify the data — either check every record, or use a documented random-sampling method — and correct any errors found before analysis.
5. Analyse the verified data for trends against the hospital's benchmark or acceptable quality level. Where a benchmark isn't yet set, base it on national/international norms.
6. Where a benchmark is not met, or the trend is unfavourable, raise it for a quality improvement project. Where a benchmark is consistently met, monitoring frequency may be reduced.
7. Evaluate the outcome of any improvement made — did the rate actually move?
8. Communicate rates, trends and improvement opportunities to staff — including positive feedback, not only problems — via bulletin, newsletter or meeting.

5. Records

Supported by Records 1-8 of the PSQ.3 Required Records pack — the indicator definition sheets, indicator data logs, and the data verification, analysis and feedback records.

6. Related Documents

Policy on Quality Indicators, Data Analysis and Improvement
(HCO/PSQ/POL/03) — PSQ.3 policy

7. Review

Reviewed annually, or immediately if a data-quality issue reveals a gap in this procedure.

8. References

US Centers for Disease Control and Prevention, National Healthcare Safety Network. (2026). NHSN Patient Safety Component Manual — device-associated event definitions (CAUTI, CLABSI, VAP) and procedure-associated event definitions (SSI). <https://www.cdc.gov/nhsn/>